

WITHDRAWAL ASSESSMENT TOOL

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WHAT IS WITHDRAWAL SCORE ASSESSMENT TOOL

WAT is an assessment instrument for monitoring opioid and benzodiazepine withdrawal symptoms in Pediatric patients where repeated observations during analgesia and sedative weaning takes place.

A prolonged exposure to opioid exposure ,children may develop opioid –induced hyperalgesia ,tolerance and withdrawal

WHEN TO START WAT ASSESSMENT

- **WAT scoring tool will be initiated when patients are started on scheduled dose of sedation / an attempt to wean from narcotics and benzodiazepine continuous infusions.**

HOW TO DO WITHDRAWAL ASSESSMENT TOOL

SEDATION WITHDRAWAL SCORE CHART

- Score the child every 6 hours, based on the child's behaviour during the preceding 6 hours
- Each category is scored **0, 1 or 2** Symptom absent = 0; Symptom mild = 1; Symptom severe = 2
- The total score will be in the range 0 – 24 and will help guide subsequent actions

[illegible]

Step -1 Review WAT familiarizing with the tool.

Step-2. Review nursing documentation from the previous 12 hours and score.

**LOOSE/watery stools: Score 0 if none, score 1 if Symptoms mild , Score 2 If severe symptoms.*

**Vomiting /gagging: Score 0 if none, Score 1If mild, Score 2 If severe symptoms*

**Temperature less than 37.8:Score 0,Score 1 if temperature more than 37.8 *C ,Score 2 If Temp more than 39*C ,for the majority of the 12 hour time period.*

Step -3.2 Minutes pre-stimulus observation .

**State: Score 0, If the child is asleep/awake and calm/cooperative .Score 1, If the child is awake and distressed during the 2 minutes prior to the stimulus, Score 2 If severe symptoms present.*

**Tremors: Score 0, If no/mild tremors note Score 1, If moderate ,Score 2 severe tremors noted.*

**Sweating: Score 0, If sweating is absent.*

Score 1 If Mild Sweating present. Score 2 If severe Sweating present.

**Uncoordinated /Repetitive movements : Score 0, If normal movement ,Score 1 If only mild unco-ordinated or repetitive movement ,Score 2 if have severe symptoms.*

Step -4.1 Minutes Stimulus observation

During normal care, stimulate the patient by calling their names in a calm voice. If no responds gently touch the child. If there is still no response assess the child during planned noxious procedure ,such as repositioning, suctioning.

**Startle to touch : Score 0, If no or only mild
startle.*

*Score 1, if moderate startle occurs when
touched during the stimulus. Score 2 If severe
symptoms present.*

**Muscle tone : Score 0, If normal tone*

*Score 1, If tone increased mild
during the stimulus.*

*Score 2, If severe symptoms
present.*

- ***Step-5.**Post -stimulus recovery*

Score0,If the child calm in less than 2 minutes.

Score 1,If the time to return to calm state is 2-5 minutes .

Score2,If the time to return to calm state is greater than 5 minutes .

FREQUENCY OF WAT ASSESSMENT

- *The WAT is completed and documented at least once per shift until 48 hours after the last PRN narcotics /benzodiazepines dose.*
- *If Symptoms are severe Score the child every 6 hours.*
- *It should be documented 1-2 hours after administering a rescue dose of narcotics or benzodiazepines.*

- *More frequent assessment may be necessary in very symptomatic patients and will require communication order by the prescribing doctor.*

HOW TO GET A SCORE?

- *Sum up all of the points scored to get the WAT score.*

WHAT DOES THE NUMBER MEAN?

A higher WAT score indicates more withdrawal symptoms while a lower score indicates fewer.

WHEN TO NOTIFY THE DOCTOR

- *Notify the provider for a WAT score as indicated the doctors order.*
- *Usually the score 0-24, and will help guide subsequent actions :
At the daily review score less than 4 =Continue weaning programme, 4-6=Do not wean medications , More than 6 =Contact PICU advise.*
- *If the child has Pre-Exiting symptoms such as Tremor , diarrhea due to another cause , fever from infectious process the provider may request to be notified for higher score.*

WITHDRAWAL MANAGEMENT

- *Consider analgesia requirements-Regular paracetamol may be helpful.*
- *Continue to use sedation withdrawal score chart for 48 hours after last dose given , or until discharge home.*
- *Most Patients should be managed on a combination of oral morphine solution and clonidine .*
- *Additional doses of Chloral hydrate can be considered.*
- *If symptoms are mild , start with clonidine , if severe ,give clonidine and oral morphine ,also use oral diazepam.*

DRUG	DOSE	WEANING
ORAL MORPHINE 10mg /5ml	0.1-0.2 mg/kg 4-6 hourly. Can increase according to responds	Wean by 20%of initial dose. Weaning should be complete in 5-10 days.
CLONIDINE	Dose 0.5-2 mcg/kg 6 hourly. If orally 1-5 mcg/kg/ hours for 6-8hrs. Monitor BP and HR.	Wean by 1 mcg/kg/dose daily. Aim to wean within 5 -10 Days . If also requiring oral Morphine consider weaning drugs alternative days. Aiming to complete weaning process in 10 days.
CHLORAL HYDRATE 100mg/ml	30-50 mg/kg/dose for 4-6 hours.	If symptoms severe give along with morphine and clonidine.
DIAZEPAM ORAL	0.2 mg/kg 6 th hourly Then gradually wean .	Can use together with morphine and clonidine.

Nursing management of child with sedation withdrawal/during WAT assessment

- *1. History Collection : Essential elements of history in withdrawal syndrome includes.*
 - *Types of drugs given.*
 - * Duration of treatment/addiction.*
 - * Time of last administration.*
 - * Reason for the child cessation of drug.*
 - * Previously withdrawal symptoms and the severity.*
 - * Alternative treatments used to relieve withdrawal symptoms.*

2. Identify the signs and symptoms

Identify the signs and symptoms related to sedations withdrawal or by other symptoms related to infectious process.

3. Health education

Give health education to parent regarding the withdrawal symptoms and management.

4. Adequate sleep and rest.

**Provide calm and quite environment.*

**Avoid procedures during sleeping time as possible.*

5. Maintain adequate hydration.

**Monitor the signs of dehydration by monitoring vital signs , skin condition ,any diarrhea ,vomiting....*

6. Monitor the neurological status of the child, ,Inform to doctor if the child have any episodes of seizure or abnormal body movement.

7. Avoiding of other sedations is important for to identify the withdrawal symptoms and prompt management.

8. Gradual weaning of sedations of continuous infusions as prescribed by doctor is very important to reduce the risk/severity of symptoms.

9. Recording and reporting

**WAT assessment.*

**Weaning medications given/not.*

**Any complicated signs and symptoms.*

REFERENCES

- *1.BNF for Children 2011-2012 ; bnfc.org.*
- *Pediatric formulary (8th edition)2010:Guys and St.Thomas ,
Kings college and University Lewisham hospitals .*

THANK YOU

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